VUGA NDAKUMVA / INTERACTIVE HEALTH EDUCATION

What Are Alcohol and Drugs?

Category 01: Understanding Alcohol & Drugs • Topic 01

A complete educational lesson with clear science, everyday examples, interactive learning scripts, feedback, safety guidance and a reference register.

FOR YOUNG PEOPLE, ADULTS AND FAMILIES

Designed for accessible mobile learning and offline reading. No user account, age-selection gate, personal disclosure, or screening is needed to learn this topic.

Status: Evidence-informed educational draft; NOT clinically approved for public publication.

Prepared: 11 November 2026 | Language: English master draft | Version: 1.0

Important: The activities inside this PDF are a readable specification of future website interactions, not working website controls or a validated assessment.

01 Start here: What will you learn?

Alcohol and other psychoactive substances can change how the brain and body work. Understanding the basics helps people make informed decisions, notice risks, support others and seek appropriate care. This lesson explains the meaning of alcohol and drugs without blame or frightening exaggeration.

By the end, you should be able to:

• Explain alcohol, drugs, psychoactive substances, medicines and non-medical use in simple language.

• Identify broad groups of substances and understand that effects and risks vary.

• Distinguish using a substance, experiencing harm, and developing a substance use disorder.

• Recognize a few situations where immediate medical help matters.

• Practice responding to misinformation or peer pressure in a fictional scenario.

Estimated learning time: about 8–12 minutes for the core text; optional activities can be completed separately. This is a design estimate, not a measured completion time.

How to use this lesson: Read the essential explanation first. If you prefer, skip activities and go directly to Key Takeaways or Get Help. Future audio, video and visual formats must offer an equivalent text alternative.

For teachers and caregivers: Explain concepts without assuming a young person has already used substances. Avoid public disclosures and use fictional examples instead.

02 What is alcohol?

Alcoholic drinks contain ethanol. Ethanol is a psychoactive chemical, meaning it affects the brain and can change judgment, attention, mood, coordination and behavior. It is also toxic: alcohol consumption can damage health and, for some people, lead to dependence. Beer, wine and spirits differ in strength and serving size, but the psychoactive ingredient is ethanol. [1]

Someone may drink alcohol in a social setting and appear relaxed, but alcohol can still impair reaction time or decision-making. Harm is not limited to people with dependence. Risk can occur after a single occasion, over repeated use, or through long-term health effects. Even low consumption is not risk-free. [1]

A key distinction: alcohol is socially familiar in many settings, but common does not mean harmless. Alcohol is classified among psychoactive substances, even when it is legally sold to adults under applicable laws. [1,2]

Example: A person leaves a celebration after drinking and feels confident enough to ride a motorcycle. Their confidence does not mean their reaction time is unaffected. The safer choice is not to drive or ride while impaired.

03 What are drugs and psychoactive substances?

In healthcare, the word drug can mean a medicine used to prevent or treat illness. In everyday conversation, it often refers to substances used for non-medical effects. These meanings are not identical. A psychoactive substance affects mental processes such as perception, thinking, feelings, awareness or behavior. Alcohol, nicotine, some prescribed medicines, cannabis, opioids and stimulants can be psychoactive. [2,3]

Not every medicine is psychoactive, and not every psychoactive substance automatically causes dependence. Some medicines are effective and important when appropriately prescribed and used; they may also carry risks if misused. Legal status, medical usefulness and health risk are different questions. [2,3]

Rather than labeling people as “good” or “bad,” this programme uses neutral, accurate terms such as substance use, harmful use and substance use disorder.

04 Broad substance groups: a practical guide

Different substances work differently. The categories below are simplified for education; many substances have mixed effects, and the same substance can affect people differently.

Group	Examples	Possible effects or concerns
Alcohol	Beer, wine, spirits	Impaired coordination and judgment; poisoning and longer-term disease risk.
Stimulants	Cocaine; amphetamine-type stimulants	Increased alertness; possible agitation, cardiovascular risks and sleep disturbance.
Opioids	Some pain medicines; heroin	Pain relief or sedation; overdose can dangerously slow breathing.
Sedatives	Some prescribed sleep or anxiety medicines	Drowsiness, impaired coordination; dangerous interaction with alcohol or opioids.
Cannabis	Cannabis products	Changes in attention, perception and memory; risks vary with age and context.
Nicotine / tobacco	Cigarettes and other nicotine products	Dependence and important health risks; delivery products differ.
Inhalants	Misused glues, solvents or aerosols	Potentially severe toxic effects, including sudden medical emergencies.

These examples describe categories; they are not instructions for use. Avoid assuming a product is safe because it is “natural,” prescribed to someone else, or legally available. [2,3]

05 How can substances affect the body and brain?

Psychoactive substances can influence communication between nerve cells and alter processes involved in attention, pleasure, movement, memory, sleep or emotions. The exact mechanism and level of risk depend on the substance, amount, individual health, circumstances and whether other substances are involved. [2,3]

Short-term concerns can include impaired decisions, falls, injuries, confusion, unsafe driving, or poisoning. Longer-term concerns may include worsening physical health, problems in relationships or work, and substance use disorders. Not everyone who uses a substance develops a disorder, but adverse effects can happen without a diagnosis. [1,3]

Some medications can interact with alcohol or other substances. A pharmacist or clinician can advise on prescribed medicines; a learning website should not attempt individualized interaction checks.

06 Three ideas that are often confused

Term	What it means
Substance use	A person consumes a substance. This alone does not establish dependence or a medical diagnosis.
Harmful or risky use	Use that increases the chance of injury or contributes to health, social or other harm. Harm may occur without dependence.
Substance use disorder	A recognized health condition involving a problematic pattern of use, assessed by qualified professionals using established criteria.

Avoid conclusions based on appearances, social stereotypes, or a single behavior. A person concerned about their use can seek a confidential professional assessment. [4]

07 Real-life examples: recognize the situation

Scenario A — At a school celebration

Aline, a fictional 16-year-old learner, is offered an alcoholic drink by older students. She says no. They tell her she is not mature unless she drinks. The important issue is not proving courage; it is respecting her choice, avoiding pressure and identifying a safe adult or friend if the pressure continues.

Scenario B — After work

Jean, a fictional 32-year-old worker, drinks at the end of a stressful week and plans to ride home. He believes a short rest is enough to remove the effects. The safer lesson is that feeling alert is not a reliable measure of fitness to drive. Arrange non-driving transport and avoid riding or driving while impaired.

Scenario C — Someone else’s prescription

A friend offers Mireille a prescribed sleeping tablet because she has not slept well. Although a clinician may prescribe such medicines for specific medical reasons, it is unsafe to assume another person’s prescription is suitable. She should ask an appropriate healthcare professional for guidance.

Scenario D — Concern about a family member

An adult family member has begun missing responsibilities and seems increasingly unwell. Rather than accusing or diagnosing them, the family can describe specific observations, ask about wellbeing in private, listen, and encourage professional help. If safety is threatened, urgent assistance takes priority.

08 Myths versus facts

Common statement	Evidence-informed response
“Alcohol is not a drug.”	Alcohol is a psychoactive substance that affects the brain and body. [1,2]
“Only illegal drugs can be harmful.”	Legal substances and prescription medicines can also cause harm when used unsafely. [2,3]
“If someone has no addiction, their use is harmless.”	Injuries, poisoning and health risks can occur without dependence. [1]
“Everyone who tries a substance becomes addicted.”	Not all use leads to a disorder; risk varies and early support can matter. [2,4]
“A person with a disorder cannot recover.”	Evidence-based treatment and recovery support are available. [4]

Language matters: use “person experiencing a substance use disorder” instead of labels that reduce an individual to their condition.

09 Interactive learning: choose and learn

These are website interaction scripts presented in readable PDF form. Users will tap an option and receive feedback in the eventual application. No answer will be treated as a diagnosis; no personal information is requested.

Activity 1 — Which statement is accurate?

Question: Which sentence best explains alcohol?

A. Alcohol is harmless if it is sold legally.
B. Alcohol contains ethanol, which can affect the brain and cause harm.
C. Alcohol can only harm someone after many years.

Feedback: B is correct. Alcohol is psychoactive and toxic. Legal availability does not remove risks, and both immediate and long-term harms are possible. [1]

Activity 2 — A friend offers a pill

Fictional situation: A friend offers you a medication prescribed to them, saying it will help you relax. What is a safer response?

A. Take it because it was prescribed by a doctor.
B. Refuse the shared medication and seek healthcare advice if needed.
C. Mix it with a drink to make it more effective.

Feedback: B is the safer response. A prescription is individualized. Mixing sedatives with alcohol or other substances may be especially dangerous. Do not take another person’s medicine.

Activity 3 — Respond to peer pressure

Fictional situation: Friends pressure a student to drink at a gathering. Which response could help?

A. Join in to avoid embarrassment.
B. State a boundary, suggest another activity, and seek support if needed.
C. Stay silent because nobody should ever disagree with friends.

Feedback: B supports personal choice without judgment. Leaving an uncomfortable situation or seeking help from a safe person is also reasonable.

Activity 4 — Is it a diagnosis?

Question: Can a quiz on this website diagnose a substance use disorder?

A. Yes, one answer is enough.
B. No. Educational activities help learning; clinical assessment requires qualified care.

Feedback: B is correct. This programme teaches health literacy. It does not diagnose, prescribe medication or replace a professional assessment.

Design note: feedback should be delivered after each choice. Users can retry, skip, or read the answer. Avoid streaks, scores or badges that reward disclosure of sensitive substance use.

10 When a situation needs urgent care

Seek urgent medical assistance if a person is unconscious, cannot be awakened, has dangerously slow or irregular breathing, experiences seizures, or shows other serious signs of poisoning. Contact the appropriate emergency service or reach an emergency department immediately. Do not leave an unresponsive person alone while arranging help.

If a person may be dependent on alcohol and is considering suddenly stopping, they should discuss withdrawal risk with a qualified clinician. Alcohol withdrawal can be medically dangerous and sometimes requires supervised care. This lesson is not a detoxification guide.

If you are supporting a young person, prioritize their immediate safety and connect them with a trusted, appropriate adult or qualified professional. Avoid public disclosure or punitive humiliation.

Rwanda-specific emergency numbers and referral services must be independently verified and approved before the platform publishes them. The educational lesson does not invent an emergency contact.

This lesson does not provide medication, dosing, substance preparation, detoxification instructions or individualized medical advice.

11 A simple way to support someone

A helpful conversation can begin with specific, nonjudgmental observations: “I noticed you have seemed unwell lately, and I am concerned. Would you like to talk?” Listen, ask what support they want, and avoid threats or public accusations. Offer help finding professional assessment if needed. Do not promise secrecy when a child or another person is at immediate risk.

For adults: support does not mean controlling their every decision. For children: safeguarding obligations and age-appropriate family or professional involvement may apply.

12 Key takeaways

1. Alcohol is a psychoactive, toxic substance containing ethanol. [1]
2. Drugs may refer to legitimate medicines or substances used for non-medical effects; context matters. [2,3]
3. Legal availability does not prove safety, and not every psychoactive substance causes dependence. [2]
4. Substance use, health risks and substance use disorders are related but distinct concepts. [1,4]
5. People deserve respectful information and access to professional care without shame. [4]

What next? Explore the next lesson in “Understanding Alcohol & Drugs,” or open “Get Help & Support Someone.” No assessment is required to continue.

13 Accessibility and interactive publication notes

The following specifies how this already-written content can be delivered in the future website. These are content and accessibility decisions, not application code.

Content element	User experience / accessibility requirement
Start screen	Title, 2-sentence introduction, estimated duration, skip to help.
Core lesson	Short sections with clear headings; one main idea at a time.
Optional visual	Simple diagram of “substance → brain/body effects → possible risks”; meaningful alt text.
Audio	Professional narration plus downloadable transcript; playback not automatic.
Scenario choices	Large, labeled tap targets; selectable by keyboard; feedback communicated to screen readers.
Feedback	Explain why an option is safer or more accurate; no shame or scoring of personal disclosure.
Supporting evidence	Source/date links in a “Learn more” area; visible editorial review date.
Low-data mode	All essential information accessible as text without downloading video.
Privacy	Anonymous browsing by default; no personal-use history gathered by lesson activities.

14 Safeguarding and editorial approval checklist

• Clinical reviewer verifies health risks, emergency symptoms, dependence wording and treatment boundaries.

• Youth safeguarding reviewer checks school scenarios, language and age-appropriate examples.

• Rwanda legal/policy reviewer checks any age-specific policy, local service details and referral claims before publication.

• Kinyarwanda and French translations receive independent human review of medical meaning.

• Accessibility reviewer checks screen reader behavior, captions, contrast and reduced-motion preferences.

• Content owner records approval, review date and source version; update on material evidence or policy changes.

The source material below informs this draft, but referencing national policy is not itself a certification or a substitute for required licensing or authorization.

15 Source and evidence register

[1] World Health Organization. Alcohol. Fact sheet, 28 June 2024.
https://www.who.int/news-room/fact-sheets/detail/alcohol

[2] World Health Organization. Drugs (psychoactive). Health topic overview.
https://www.who.int/health-topics/drugs-psychoactive

[3] World Health Organization. Alcohol, Drugs and Addictive Behaviours: Terminology.
https://www.who.int/teams/mental-health-and-substance-use/alcohol-drugs-and-addictive-behaviours/terminology

[4] WHO & UNODC. International Standards for the Treatment of Drug Use Disorders, 2020.
https://www.who.int/publications/i/item/international-standards-for-the-treatment-of-drug-use-disorders

[5] WHO & UNODC. International Standards on Drug Use Prevention, Second Edition, 2018.
https://www.who.int/publications/i/item/international-standards-for-drug-use-prevention-second-edition-2018

[6] Rwanda Ministry of Health. Health Sector Strategic Plan V, July 2024–June 2029.
https://www.moh.gov.rw/fileadmin/user_upload/Moh/Publications/Reports/new_annual_Report/HSSP_V_signed.pdf

Rwanda-specific context: The national HSSP V identifies alcohol consumption among major noncommunicable-disease risk factors and recognizes gaps in mental health service access. This is a strategic policy reference, not proof that this draft has received national approval. [6]

Editorial status: Draft for professional review. Suggested next lesson: “How Alcohol and Other Substances Affect the Brain and Body.”